Outlook

RE: Urgent Concerns Regarding Policies, Patient Safety, and Accountability

From Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>
Date Tue 02-Apr-24 15:38
To Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Cc McCabe, Endreya (IHS/PHX) <Endreya.McCabe@ihs.gov>

Kevin,

I keep my calendar up to date. Please use Outlook to pick a time that's free on my calendar.

Very Respectfully,

Clinton D. Pope, MSIE, CPHQ
Chief Quality Officer
Quality Division
Phoenix Indian Medical Center
Indian Health Service

Office: (602)581-6563
Cell: (928)892-3234

IHS Mission: *To raise the physical, mental, social, and spiritual health of American Indians and Alaska Natives to the highest level*

PIMC Ascend: *Building our future together by listening, empowering, and caring for our Community*

From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Tuesday, April 2, 2024 2:03 PM
To: Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>
Cc: McCabe, Endreya (IHS/PHX) <Endreya.McCabe@ihs.gov>
Subject: RE: Urgent Concerns Regarding Policies, Patient Safety, and Accountability

Now that I'm permanently in the Policy and Procedure Nurse Consultant role, it's crucial for me to have explicit expectations outlined. Having a clear understanding of my responsibilities, performance goals, and how my success will be measured is essential. I want to proactively engage to define these expectations and ensure I'm exceeding them, contributing significantly to the quality management goals of the organization. For this reason, I haven't scheduled a follow-up meeting yet because I need a better understanding of my true role and function within the organization is. From what I understand of the position description, I am to act as an analyst and advisor, using my nursing background to consult on the development, content, and validity of policies, protocols, and guidelines. Analyze existing policies to make sure they're valid, consistent, clear, and reflect actual practice. However, there's extreme ambiguity about what happens when I find an issue. What am I supposed to do, and how do I ensure policy owners make the necessary changes? If there is no support from leadership and the executive suite than what is the point of having the position filled.
The position also involves keeping up-to-date with TJC, CMS, and other regulations, and then using that knowledge to help policy owners develop new or revised policies. Supposedly, I oversee policy

distribution, manage online resources, and advise managers and executives on implementing new regulations. So I changed workflows to ensure that I am truly the one overseeing the release of policies but no process of what to do if the policy has an issue or how to address it.
Thank you for sharing the detailed language from 25 USC §1675. I appreciate your commitment to ensuring clarity and compliance within our quality division. I am aware of the language and the tests by the courts to determine what qualifies as quality assurance in regards to 25 USC § 1675. I completely agree with your perspective on the purpose of such codes—to foster psychological safety and encourage a culture of transparency and continuous improvement. As you are already aware I agree with the need for a Just Culture. Your emphasis on the importance of whistleblowers and proactive monitoring to identify and address potential risks is commendable and aligns perfectly with our overarching goals.
I truly appreciate your acknowledgment of the importance of whistleblowers and proactive efforts. Your words are encouraging to hear. You must understand and respect that building trust takes time and action, especially given past experiences. I'm committed to creating a space where concerns can be raised constructively and without fear of retaliation.
I think the next logical step would be first for me to meet with you and Endreya to discuss the expectations, performance goals and how they will be measured. Then we can determine a plan on how to establish a structured process to systematically enhance the organization's policy system.

Please provide me a good time to schedule a meeting to discuss.

V/r

Hon. Kevin D. Turnage, MSN Ed., RN, AMB-BC™, NE-BC®, NHDP-BC™, NPD-BC™
Policy and Procedure Nurse Consultant, COR II,
You can now book and manage appointments using our booking page.
https://outlook.office365.com/owa/calendar/KevinTurnage1@ihsgov.onmicrosoft.com/bookings/

From: Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>
Sent: Tuesday, April 2, 2024 12:07
To: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Cc: McCabe, Endreya (IHS/PHX) <Endreya.McCabe@ihs.gov>
Subject: RE: Urgent Concerns Regarding Policies, Patient Safety, and Accountability

Kevin,

When do we meet next? I'm actually really excited to create a system of continuous improvement in policy management.

See attached. This is the exact wording of the US Code…

I was too inclusive to say “everything we do in the quality division...” However, it does include much more than only “data.” Specifically:

“The term “medical quality assurance program” means any activity carried out before, on, or after March 23, 2010, by or for any Indian health program or urban Indian organization to assess the quality of medical care, including activities conducted by or on behalf of individuals, Indian health program or urban Indian organization medical or dental treatment review committees, or other review bodies responsible for quality assurance, credentials, infection control, patient safety, patient care assessment (including treatment procedures, blood, drugs, and therapeutics), medical records, health resources management review, and identification and prevention of medical or dental incidents and risks.”

Which is inclusive of compliance to standards and policies, which are quality assurance activities. Not all policies fall into this “medical quality assurance program,” but a great number of them do.

From my perspective and understanding, the reason this and similar codes exist it to support psychological safety (elimination of fear) by creating assurances that there are no barriers to discussing and transparently seeing quality and safety issues within an organization. In other words, I need people to speak up more, in such a way that it leads to continuous improvement. I want the whistleblowers to help me see the issues, so they can be fixed. I want audits and monitoring, so we can identify risks and issues before they lead to adverse events.

Thanks,

Clint

Very Respectfully,

Clinton D. Pope, MSIE, CPHQ
Chief Quality Officer
Quality Division
Phoenix Indian Medical Center
Indian Health Service

Office: (602)581-6563
Cell: (928)892-3234

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From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Monday, April 1, 2024 9:18 PM
To: Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>
Cc: McCabe, Endreya (IHS/PHX) <Endreya.McCabe@ihs.gov>
Subject: Re: Urgent Concerns Regarding Policies, Patient Safety, and Accountability

Dear Clint,

I appreciate your prompt response and am glad to read your responses.

Thank you for highlighting the importance of confidentiality. I understand the need for discretion, especially given the agency's history of retaliation, including against whistleblowers such as myself. I share your

concerns and want to work with you to change that culture, which is partly why I contacted you directly and did not include Deb Ward Lund. I am attempting to open the lines of communication that are non-existent in the current state. I want to clarify that the information in my previous email wouldn't fall under U.S. Code Title 25, CHAPTER 18 SUBCHAPTER VI §1675. Section (i) of this code also clarifies that this statute does not protect information outside of specific medical quality assurance records. This law aims to foster open and honest participation in quality improvement processes within healthcare. It does so by offering protection for these quality assurance programs' discussions, documents, and findings generated explicitly.

Under 25 USC 1675, quality assurance data refers to records, analyses, and discussions generated within a formally structured medical quality assurance (MQA) program. Here's what typically constitutes quality assurance data:

Types of Data:

- **Incident Reports:** Documentation of adverse events, near-misses, medical errors, or any occurrences that could impact patient care.
- **Peer Reviews:** Evaluations of a healthcare provider's performance by their colleagues, often focusing on specific cases or procedures.
- **Root Cause Analyses:** Systematic investigations to identify the underlying factors that contribute to adverse events.
- **Committee Meeting Minutes:** Records of discussions, findings, and recommendations from MQA committees.
- **Trend Analyses:** Statistical reports and data visualizations that track patterns related to patient outcomes, errors, or improvement efforts over time.

Critical Requirements for Data to be Considered "Quality Assurance":

1. **Formal MQA Program:** The data must originate from a structured medical quality assurance program established by a healthcare facility or authorized entity. This program requires specific goals, methods, and participants.
2. **Scope of Review:** The data must relate to efforts to assess and improve the quality of healthcare services.

What is NOT Considered Quality Assurance Data:

- **Informal Conversations or Notes:** Casual discussions or personal notes taken outside of a formal MQA process, such as the conversations referenced in my email, would not likely be protected.
- **Information Unrelated to Quality Improvement:** Data not directly linked to assessing or improving healthcare quality would not fall under this protection.

Given the agency's unfortunate history of retaliation and limiting my access to saved documents, to prevent those outside the agency from learning of illegal behavior, it is imperative to adopt a cautious approach to record-keeping. So, I sent a copy to my email address. This communication is about concerns that the agency may put in a different light or deny, and this is solely for record-keeping to a secure encrypted email address. No PII or PHI is being sent, nor is there any information about the agency’s quality assurance data. This measure aims to safeguard the integrity and accessibility of pertinent information, particularly considering past challenges. Unfortunately, as an Employer, you may have misinterpreted the confidentiality provisions of 25 USC 1675 and tried to discourage employees from reporting externally, leading to confusion about reporting rights. Employees still have the right and often the obligation to report serious events or concerns to regulatory agencies. 25 USC 1675 aims to improve internal quality processes, not to stifle necessary reporting. I welcome a formal legal review of the statute by the OIG and OSC, if required, to get clarification of what constitutes a quality assurance program document and what does not.

I am fully committed to supporting efforts related to assisting with the possible UAC, as discussed by Deb Ward Lund. However, that is not what I was referring to. I was explicitly told, “Strange, I was given the information that you were assisting him with this training.” When I inquired by whom, they specifically said you had told them.

I'm committed to protecting sensitive information and fostering a safer, more transparent working environment. I am eager to collaborate with you to implement changes aimed at fostering a just culture and enhancing policy and procedure compliance within our organization. Through our collective efforts, we will strive to improve patient safety and ensure that our practices consistently meet the highest standards. I look forward to our partnership in achieving these important objectives.

V/r

Hon. Kevin D. Turnage, MSN Ed., RN, AMB-BC™, NE-BC®, NHDP-BC™, NPD-BC™
Nurse Consultant, Policy and Procedure Officer, COR II,
Phoenix Indian Medical Center
Cell: 602-647-9126

"The single greatest impediment to error prevention in the medical industry is that we punish people for making mistakes." Dr. Lucian Leape

I am not authorized to obligate the government for any work, service or product. Please do not perform any work that will require payment without a valid contract, issued by a government Contracting Officer. Changes to a contract can only be made by the contracting officer. Contractors are reminded to promptly notify the contracting officer of any conduct that may be regarded as a change to the contract in accordance with FAR52-243-7

From: Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>
Date: Monday, April 1, 2024 at 16:10
To: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Cc: McCabe, Endreya (IHS/PHX) <Endreya.McCabe@ihs.gov>
Subject: RE: Urgent Concerns Regarding Policies, Patient Safety, and Accountability

Kevin,

Please see my responses in blue below. I see that you copied your personal email. Please be aware that

everything we do in the quality division is confidential and protected under U.S. Code Title 25 CHAPTER 18 SUBCHAPTER VI §1675. No portion of the work we do should be videotaped, photographed, copied, or otherwise duplicated, or distributed except as permitted by law. I need your help to improve the organization and to eliminate fears of discussing concerns or issues. I love the improvement work we are involved in in the quality division. I'm committed to it in every way, to improve staff and patient safety. It's an arduous journey that never ends.

Thanks,

Clint

Very Respectfully,

Clinton D. Pope, MSIE, CPHQ
Chief Quality Officer
Quality Division
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From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Monday, April 1, 2024 1:27 PM
To: Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>
Cc: McCabe, Endreya (IHS/PHX) <Endreya.McCabe@ihs.gov>; Kevin Turnage <kdturnage@protonmail.com>
Subject: Urgent Concerns Regarding Policies, Patient Safety, and Accountability

Dear Clint Pope,

I am writing to share some concerns from recent discussions within our organization regarding policies, patient safety, accountability, and communication. As a registered nurse, I have a solemn duty to advocate for patients' well-being and raise concerns about any factors that could compromise their care.

During our recent conversation with you as Chief Quality Officer (CQO), certain remarks raised concerns about our approach to patient safety. Specifically, the notion that "Good Enough" suffices concerning policies affecting patient safety is troubling. Such a mindset places our patients at risk and fails to uphold the Phoenix Indian Medical Center's core values. You grossly misunderstand my meaning if you believe I would say things are “good enough” regarding risks to patient safety. The concept I intend to convey is that “perfect is the enemy of excellence,” meaning that we have to be honest about where we are and the journey it will take to improve and that we can’t let the idea of perfect prevent us from making that journey. My goal is always that of a continuous improvement journey, with no satisfaction of the status quo, because we can always do better.

Moreover, the observation that policies are allowed to expire without urgency or proactive measures to prevent it is alarming. Never at any point did I say that policies should be allowed to expire without urgency or proactive measures to prevent it. Patient safety should never be compromised due to administrative delays or oversights. Also, the suggestion that only supervisors can make sure policy compliance is concerning. I’m not sure what this is saying about supervisors… anyone at PIMC can provide input on policy compliance. My recent training on Just Culture and Psychological Safety highlights this principle of everyone having a voice and eliminating any fear that would prevent the discussion of concerns or issues. A bad precedent is set when patient safety, life safety, and environmental care policies can expire without consequence. This lax attitude increases the risk of preventable harm to patients and staff and could damage the facility's reputation. Policies don’t simply “expire.” When a policy is due for review, it is still in effect until it has been reviewed and updated as needed. There is no intention to have a “lax attitude” toward policy management. On the contrary, I have asked that we meet regularly to promote policy management and iteratively improve. As leaders, it's our collective responsibility to ensure all staff members uphold the highest standards of safety and professionalism. When policy owners consistently allow policies to expire without consequences, it sets a precedent that undermines the importance of adherence. Accountability must be maintained at every level. What type of “consequences” do you recommend? What type of countermeasures do you recommend to improve accountability? These are the exact types of questions I want to discuss as we regularly meet.

I wish to reaffirm my dedication to patient-driven care and safety. Throughout my tenure, I've consistently advocated for evidence-based changes to enhance patient outcomes. However, witnessing these efforts disregarded and flawed processes perpetuated is disheartening.

Unfortunately, previous attempts to address similar concerns have resulted in instances of retaliation and discrimination by the agency, creating a challenging environment for those advocating for necessary changes. We must foster an environment where concerns can be raised without fear of reprisal, ensuring open communication and collaboration to improve patient care. I couldn't agree more, that is the exact culture I've been promoting.

While addressing these issues may necessitate challenging conversations and systemic adjustments, we must act decisively to safeguard our patients' well-being. We owe it to them and our community to confront these concerns head-on and implement necessary changes promptly. This is why I've asked that we meet regularly to discuss policy management.

Also, I'd like to discuss the dissemination of information about my involvement in tasks or initiatives without my prior knowledge or consent. This practice must be revised to maintain transparency and professional integrity and avoid potential misunderstandings within the organization. I ask that all communications about my role be conducted transparently and respectfully. I can only assume that this is in reference to the comment made by Ms. Ward Lund that you had experience working with CPI. From my understanding, no task has been asked of you other than to communicate your experiences with Brandon Parker. I have not communicated any expectations contrary to that.

I would also like to discuss any tasks or initiatives that involve me directly, making sure I can provide relevant input and support. Clear communication and collaboration are crucial for effective teamwork.

Lastly, I sincerely hope that addressing these concerns will not result in repeated retaliation, harassment, or discrimination. Building a culture of open communication and accountability necessitates trust and respect for all team members.

I trust these concerns can be addressed promptly and effectively to uphold the Phoenix Indian Medical Center's safety and accountability standards. To ensure situational awareness, I have copied Endreya McCabe, my direct supervisor, on this email. As has been my intent, we must meet regularly to discuss these concerns and iteratively improve related processes and systems.

Thank you for your attention to these matters. I remain committed to collaborating towards a safer and more accountable future for the Phoenix Indian Medical Center.

Sincerely,

Hon. Kevin Turnage MSN Ed. RN, AMB-BC, NE-BC, NPD-BC, NHDP-BC

Nurse Consultant Policy and Procedure Officer

Cell: 602-647-9126

CC: Endreya McCabe - Director of Quality and Performance Improvement

"The single greatest impediment to error prevention in the medical industry is that we punish people for making mistakes." Dr. Lucian Leape

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